Berkeley Haas



MBA297A –HC in 21st Century: Overview on HC Structure & Cost

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Goals for this session

- Start exploration of the structure of healthcare systems – lay out the building blocks for rest of our course flow
- Understand key factors driving health system performance - look at different countries
  - Hear from someone from the UK's NHS – our 1st guest Susannah Howard!



Dimensions Often Used to Understand & Compare Health Systems

- Role of government
- Who is covered and how is care financed?
- What is covered (services, products) and under what conditions. **Where is the NO?**
- How is the delivery system organized and financed/paid?
- How are **costs contained**? Is care affordable?
- How important is **equity, access**? How try to achieve?
- How are **new technologies** deployed, used?
- How is quality measured, promoted?
- What are the system's **outcomes** in terms of health, access, equity, cost, efficiency, innovation, etc?

Other Key Dimensions/Questions…more dynamic

- How does **money flow** and where does it **follow value creation** (and where not)?
  - How is that changing?
  - What is a meaningful **time horizon**?
- Which entities should/need to interrelate, how, and **what is the leverage** each side might have to work with?
  - Can that be "disrupted"
- How do new products/service achieve market access? What does **"true" access rely on** vs regulatory access?
- What are key pressures, challenges or areas of opportunity?
- How **politicized** is the discussion and decision-making process for healthcare activities and investment?
- Level of trust in government, key health agencies, clinicians, tech?

Words often used to describe the US healthcare system – some claim it isn't a system….

- Complicated
- Fragmented/varied
- Expensive
- Inequitable
- Innovative
- Focused on care (often acute) and not health
- Lack of shared commitment



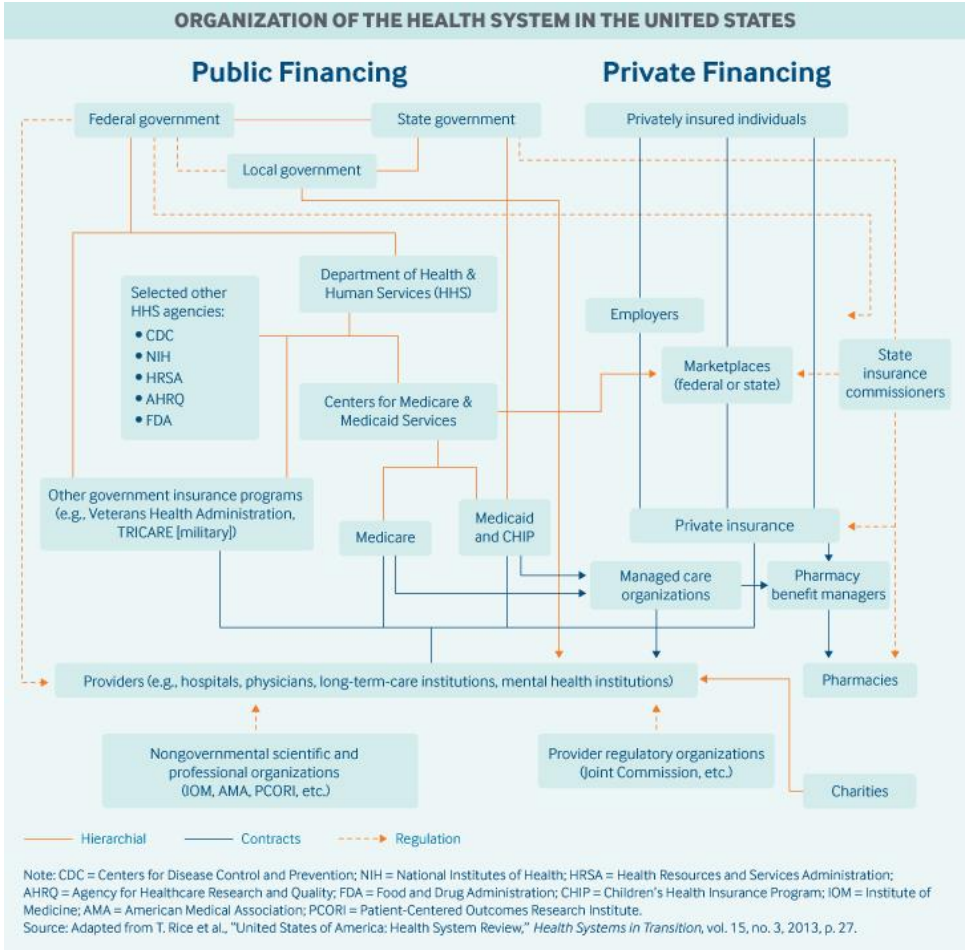
Some of the most surprising aspects of the US system through the lenses of 10 observers from Australia, France, Germany, the Netherlands, New Zealand, Norway, and England?



- Healthcare as Privilege, Not a Right
- The Price Problem
- No Accountability OR Shared Principles

https://www.healthaffairs.org/do/10.1377/hblog20190826.181413/full/

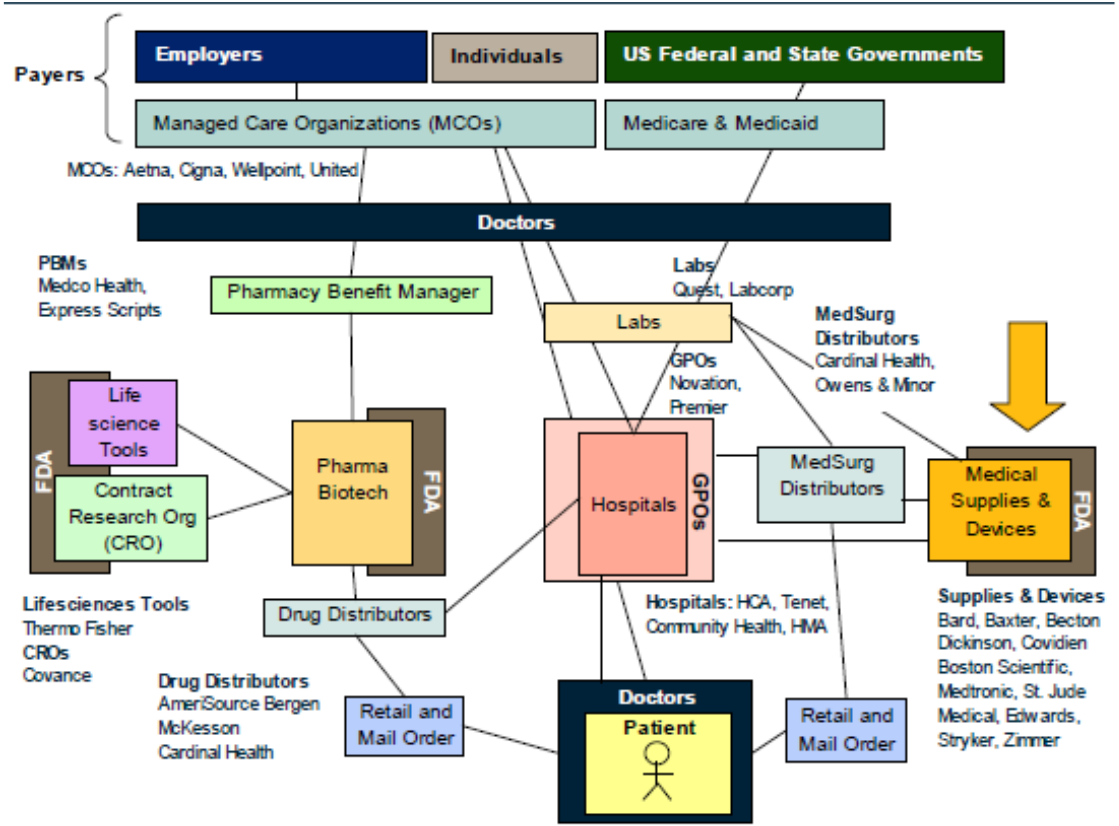
Organization of the Health System in the US



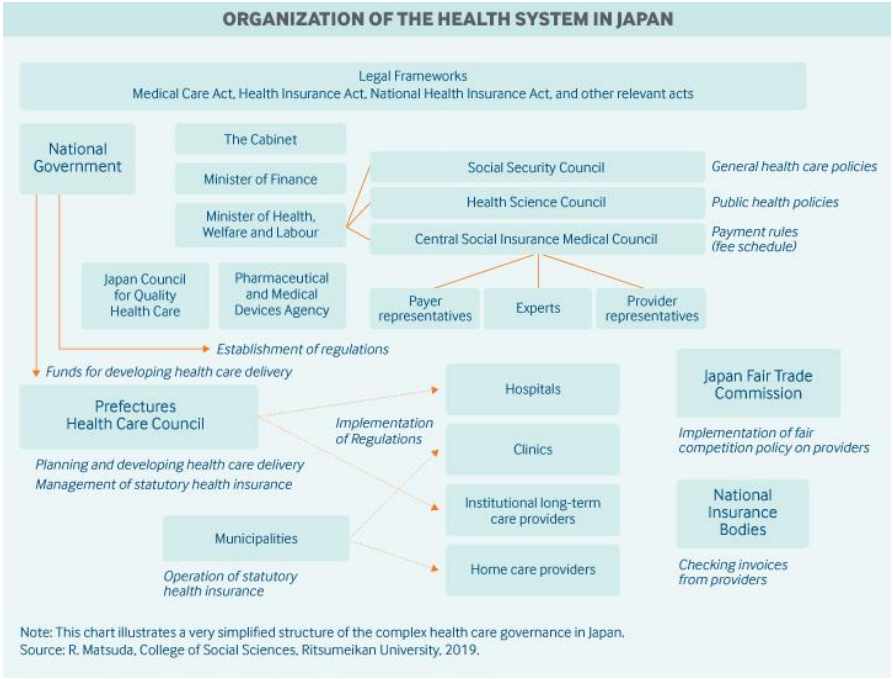
- Can see many of the building blocks we will cover in course
  - Focus here heavily on financing, regulatory
- Can see the multiplicity of "buckets" that characterizes how US manages coverage
- Not very centered on the people who will need/use the system or public health or any products

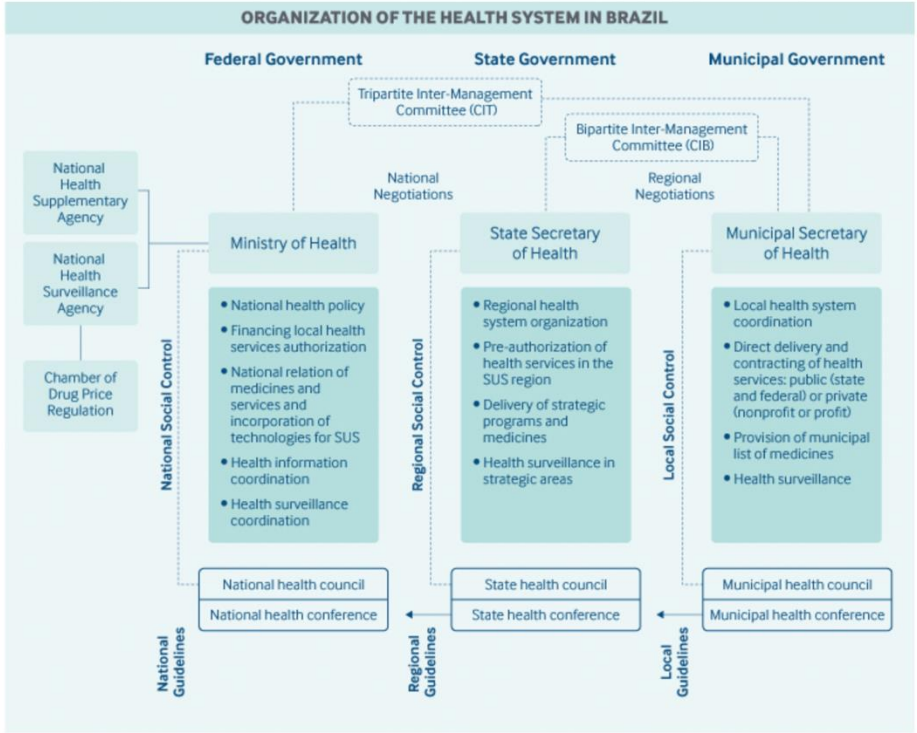
What if this picture is how it was presented?

Another viewpoint of the healthcare system would also include the product sectors and key ‘intermediaries”



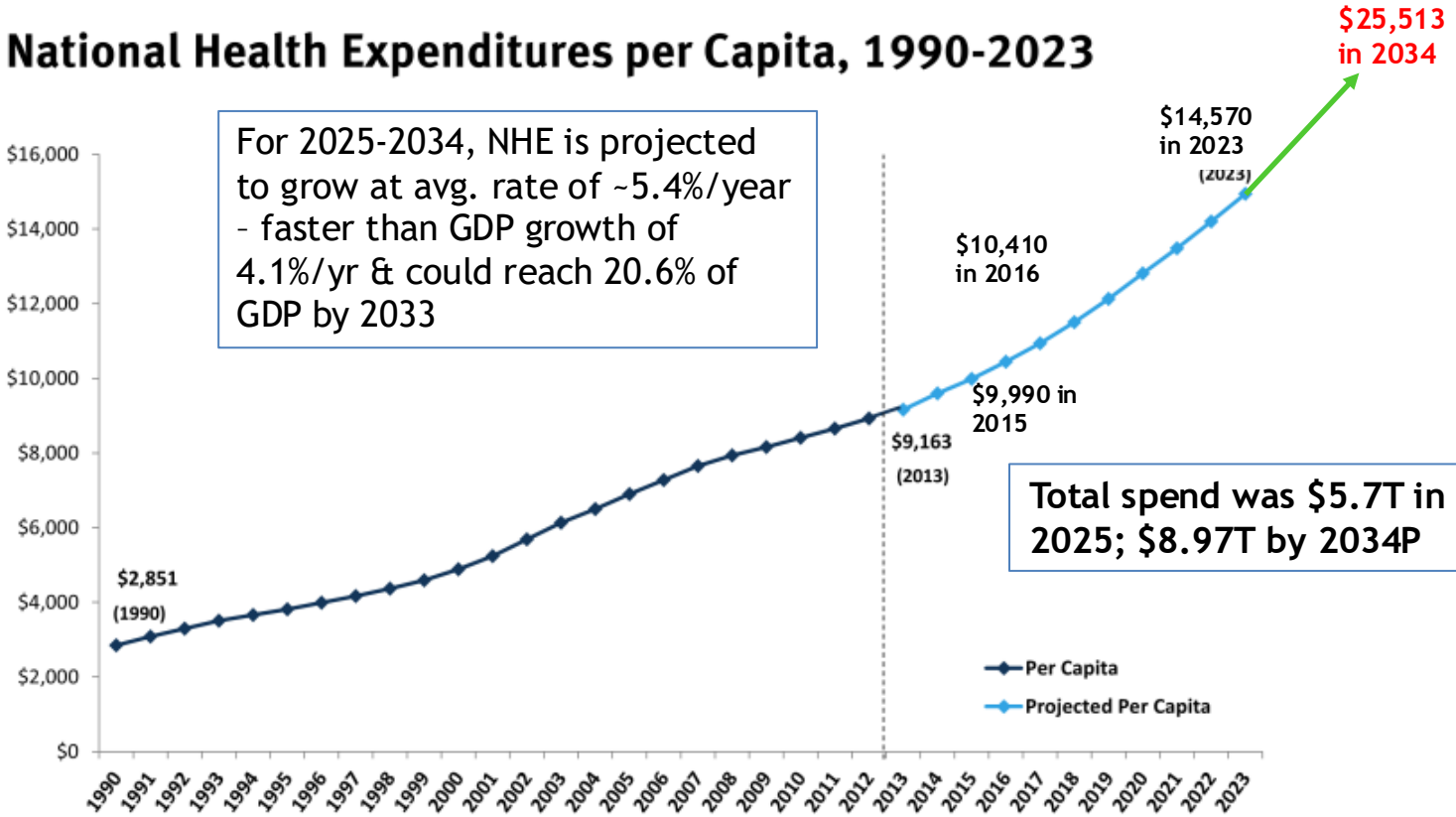
System structure varies by country - many implications



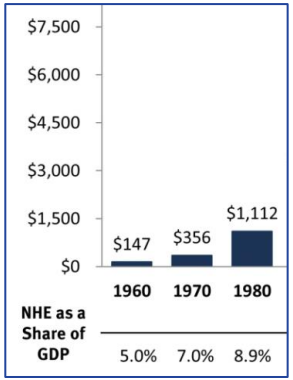


https://www.commonwealthfund.org/international-health-policy-center/countr

National Health Expenditures per Capita, 1990-2023



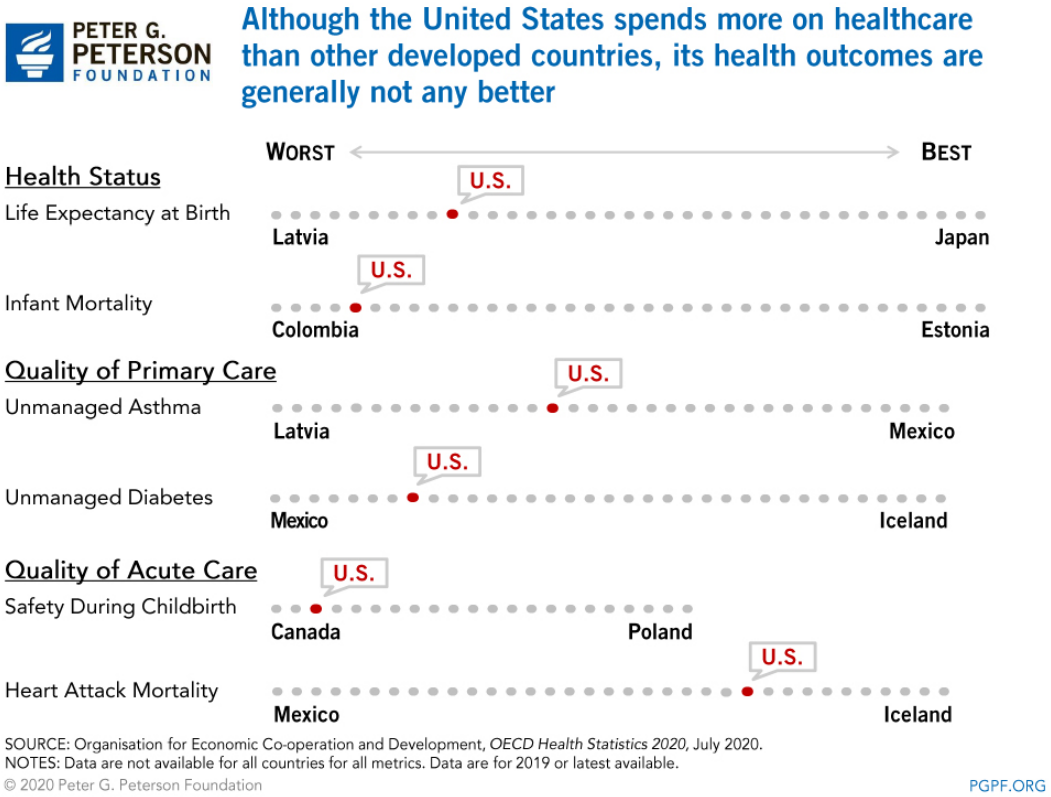
Way Back…



SOURCE: Kaiser Family Foundation calculations using NHE data from Centers for Medicare and Medicaid Services, Office of the Actuary, National Health Statistics Group, at http://www.cms.hhs.gov/NationalHealthExpendData/ (Historical data from National Health Expenditures by type of service and source of funds, file nhe12.zip; Projected data from NHE Historical and projections 1965-2023, file nhe65-23.zip).

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US spends a lot, but do they get more?

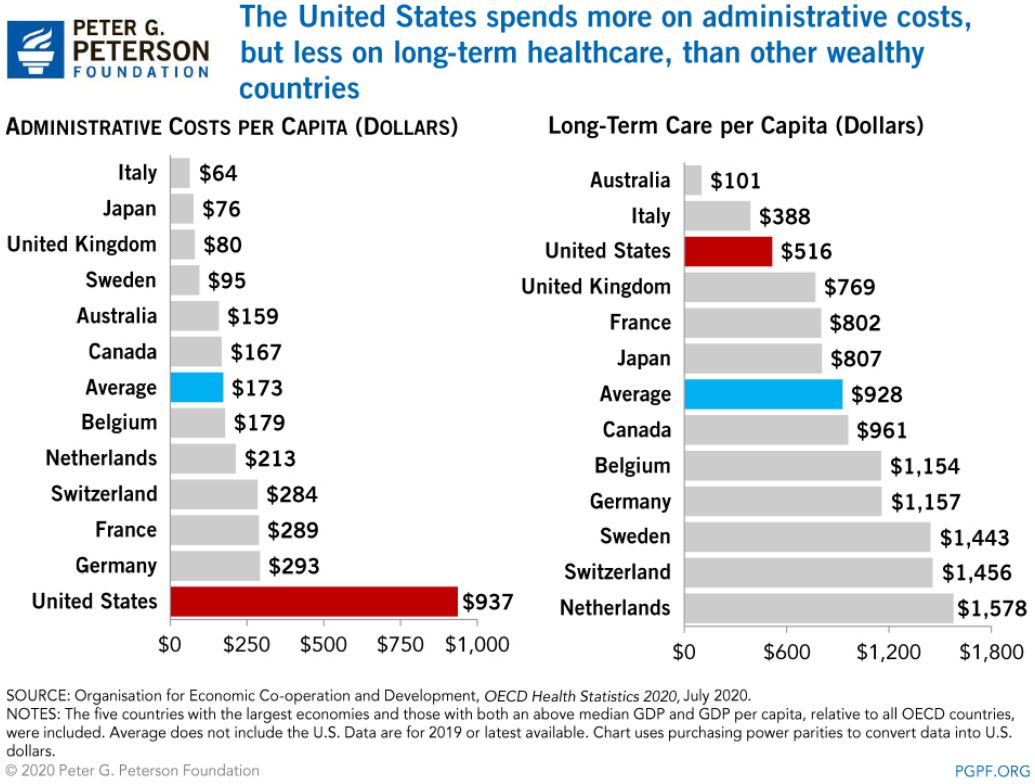


- Some outcome measures show US has lower value for how much it spends
- Have tried to peg spend to value or outcomes – change incentives and culture
  - Slow process
  - Many interfering forces
  - **Variable interpretations - what is value? Who gets to decide?**

https://www.healthsystemtracker.org/chart-collection/health-spending-u-s-compare-countries/#item-start

Why Do Comparative Analysis?

- Insights into one's own system
- Benchmark analysis
  - 1-1 or larger cohort such as OECD
- Interpret outcomes, consider proposed alternatives
- Learn new approaches



A Tale of Two Health System Comparisons

Commonwealth Fund Mirror, Mirror 2024

Health Care System Performance Rankings

	AUS	CAN	FRA	GER	NETH	NZ	SWE	SWIZ	UK	US
Overall Ranking	**1**	**7**	**5**	**9**	**2**	**4**	**6**	**8**	**3**	**10**
Access to Care	9	7	6	3	1	5	4	8	2	10
Care Process	5	4	7	9	3	1	10	6	8	2
Administrative Efficiency	2	5	4	8	6	3	7	10	1	9
Equity	1	7	6	2	3	8	–	4	5	9
Health Outcomes	1	4	5	9	7	3	6	2	8	10

Note: SWE overall ranking calculation does not include Equity domain. See "How We Conducted This Study" for more detail.

Data: Commonwealth Fund analysis.

A Tale of Two Health System Comparisons

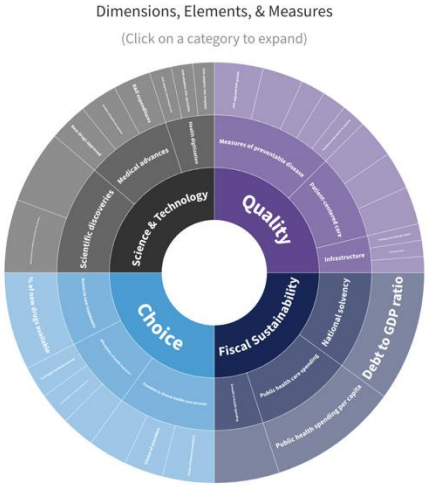
2024 World Index of Healthcare Innovation

Overall & Dimension Rankings

(Click on a column to sort)

Overall Rank	Country	Overall Tier	Overall Score	Quality	Choice	Science & Technology	Fiscal Sustainability
1	**Switzerland**	Elite	66.19	69.56	66.98	62.28	65.96
2	**Ireland**	Elite	59.52	67.52	54.19	44.93	71.45
3	**Germany**	Elite	58.54	53.12	68.55	47.77	64.71
4	**Netherlands**	Elite	57.82	62.75	56.61	49.55	62.38
5	**Norway**	Good	53.94	67.97	47.76	44.02	56.02
6	**Denmark**	Good	53.79	63.01	48.22	54.13	49.81
7	**United States**	Good	52.96	60.02	57.07	78.63	16.12
8	**Sweden**	Good	52.23	70.46	49.17	48.83	40.47
9	**Australia**	Good	52.00	66.34	64.55	34.26	42.87
10	**Belgium**	Good	51.90	57.33	48.82	45.61	55.82
11	**United Kingdom**	Good	50.63	64.44	55.29	51.52	31.26
12	**Israel**	Good	50.62	52.31	49.93	43.76	56.48
13	**Singapore**	Good	48.12	57.83	48.82	41.18	44.64
14	**Finland**	Good	47.87	68.52	41.25	40.51	41.19
15	**Taiwan**	Good	47.69	57.86	62.12	17.42	53.37
16	**Hong Kong**	Good	47.59	52.63	58.51	29.96	49.25
17	**France**	Moderate	46.62	61.62	54.81	40.21	29.83
18	**Canada**	Moderate	45.96	64.96	50.15	32.80	35.93
19	**Spain**	Moderate	45.11	56.65	53.94	30.24	39.61

Source: FREOPP World Index of Healthcare Innovation
Analysis: Gregg Girvan & Avik Roy; Graphic: Avik Roy

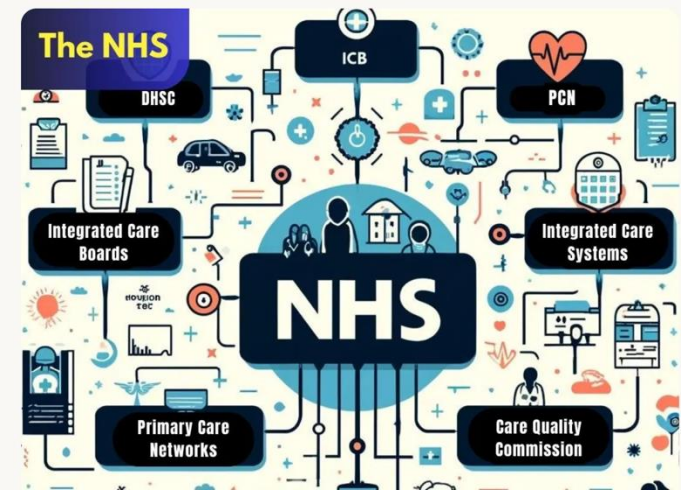


2024 vs. 2020	Country	2024 Tier	2024 Rank	2022 Rank	2021 Rank	2020 Rank
✤ 0	**Switzerland**	Elite	1	1	1	1
⬆ 3	**Ireland**	Elite	2	2	4	5
⬇ 1	**Germany**	Elite	3	4	3	2
⬇ 1	**Netherlands**	Elite	4	3	2	3
⬆ 7	**Norway**	Good	5	14	24	12
⬆ 8	**Denmark**	Good	6	7	11	14
⬇ 3	**United States**	Good	7	11	6	4

Key influences on cost trend – what can things can a country try to control besides price?

- Aging population's chronic disease/disability
- Coverage expansion and contraction; other regulatory and private sector impact on payment reform
- Economic factors & regulatory/tax reform
- Use and intensity of services/products
  - Addition of social and mental health services as rising new one
  - Barriers to access
- Efficiency, productivity, waste/fraud - new initiatives/technologies, esp w/ AI
- Predictive analytics – identify cost potential and intervene earlier to minimize







Susannah Howard

Coming up next

- Review survey data
- Start next module: The Money Stack!
- Healthcare finance & coverage section
  - Insurance 101, major public and private forms of coverage
    - Close up on Medicare Advantage
  - Employers, households and consumerism

